BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, NET response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

Check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver	FBC, renal/liver tests, BUN and urine protein	FBC, kidneys and liver every 4 weeks	Earlier assessment if unwell, dehydrated or less urine	Review abnormalities before continuing	Follow unresolved effects
Glucose / blood fats	Fasting glucose, cholesterol and triglycerides	Lipids regularly; fasting glucose as clinically indicated	Report thirst, frequent urine or marked fatigue	Review treatment for diabetes or lipid changes	Individual follow-up
Infection / mouth / lungs	Review active infection; hepatitis B surface antigen and core antibody	Ask about ulcers, cough, breathlessness, infection and rash	New breathing or infection symptoms need prompt contact	Assess lung inflammation versus infection	Follow symptoms after a hold or stopping
NET response / hormone plan	Document tumour site, grade, symptoms and baseline assessment	Scans / markers only as individually scheduled	Track symptoms and separate hormone medicines	Review benefit, burden and personal goals	Agree next treatment and follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00320 v6 · current Irish regimen · EMA product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Everolimus · neuroendocrine tumours: treatment and checks

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c.

Take the prescribed everolimus dose once daily every day, at the same time and consistently with or without food. There is no planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00320	FBC, renal/liver tests, BUN and urine protein	FBC, kidneys and liver every 4 weeks	Earlier assessment if unwell, dehydrated or less urine	Review abnormalities before continuing	Follow unresolved effects
Glucose / blood fats NCCP 00320	Fasting glucose, cholesterol and triglycerides	Lipids regularly; fasting glucose as clinically indicated	Report thirst, frequent urine or marked fatigue	Review treatment for diabetes or lipid changes	Individual follow-up
Infection / mouth / lungs NCCP 00320	Review active infection; hepatitis B surface antigen and core antibody	Ask about ulcers, cough, breathlessness, infection and rash	New breathing or infection symptoms need prompt contact	Assess lung inflammation versus infection	Follow symptoms after a hold or stopping
NET response / hormone plan NCCP 00320	Document tumour site, grade, symptoms and baseline assessment	Scans / markers only as individually scheduled	Track symptoms and separate hormone medicines	Review benefit, burden and personal goals	Agree next treatment and follow-up

What each check is for

1

Daily everolimus

Continuous

Everolimus is a daily targeted tablet used to help control particular advanced NETs. It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00320

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37